

METROPOLITAN GENERAL HOSPITAL

2100 Healing Way, Boston, MA 02115 | (617) 555-0800 | www.metrogen.org

DISCHARGE SUMMARY

Patient Name: Jamal Washington	DOB: May 01, 1994 (Age 74)
MRN: MRN-8531380	Gender: Female
Admission Date: June 20, 2025	Discharge Date: May 16, 2025 (10 days)
Attending Physician: Dr. Angela Forsythe, MD — Cardiology	Insurance: Cigna #CIG-667-88901

DIAGNOSES

Primary Diagnosis	ICD-10 Code	Status
Gastroesophageal Reflux Disease (GERD)	K21.9	Chronic / Pre-existing
Community-Acquired Pneumonia	J18.9	Resolved
Osteoarthritis of Knee	M17.9	Chronic / Pre-existing
Acute ST-Elevation Myocardial Infarction (STEMI) — Anterior Wall	I21.09	Resolved

HOSPITAL COURSE

Jamal, a 74-year-old female with a known history of Type 2 Diabetes and Hypertension, presented to the Emergency Department on June 20, 2025 via ambulance with a 2-hour history of severe substernal chest pain radiating to the left arm, diaphoresis, and nausea. Initial ECG demonstrated ST-segment elevation in leads V1–V4 consistent with anterior STEMI. Troponin-I on arrival was critically elevated at 16.6 ng/mL. The patient was taken emergently to the cardiac catheterization laboratory where coronary angiography revealed 82% occlusion of the left anterior descending (LAD) artery. Successful percutaneous coronary intervention (PCI) was performed with deployment of a drug-eluting stent (Xience Skypoint 3.0 x 28mm). Post-procedural TIMI flow was grade 3. The patient was transferred to the Cardiac Care Unit (CCU) for monitoring. Serial echocardiography on day 2 showed an ejection fraction of 44%, mild anterior wall hypokinesis. Mild acute kidney injury (Creatinine 1.7 mg/dL) was noted, attributed to contrast nephropathy; this resolved with IV hydration by day 4. Blood glucose remained elevated throughout admission and endocrinology was consulted to optimize insulin regimen. The patient remained hemodynamically stable throughout and was discharged on day 10 in stable condition with close outpatient follow-up arranged.

VITALS AT DISCHARGE

BP	HR	SpO2	Temp	RR	Weight
112/66 mmHg	83 bpm	97% (RA)	97.9°F	20/min	102 kg

DISCHARGE MEDICATIONS

Medication	Dose	Frequency	Duration
Aspirin	81 mg	Once daily	Indefinite
Clopidogrel (Plavix)	75 mg	Once daily	12 months
Atorvastatin	80 mg	Once at bedtime	Indefinite
Metoprolol Succinate	50 mg	Once daily	Indefinite
Lisinopril	10 mg	Once daily	Indefinite
Insulin Glargine (Lantus)	19 units	Once daily at bedtime	Ongoing — follow with Endo
Metformin	500 mg	Twice daily with meals	Ongoing

FOLLOW-UP PLAN

Cardiology: Dr. Forsythe — 1 week post-discharge. Repeat Echo at 6 weeks.
Endocrinology: Dr. Patel — 2 weeks post-discharge for diabetes management.

Primary Care: Dr. Kim — 4 weeks. Renal function panel to confirm AKI resolution.
Cardiac Rehab: Enrollment arranged — starts July 02, 2025 at MetroGen Rehab Center.
Labs: BMP, HbA1c, Lipid Panel — 2 weeks post-discharge.

Attending Physician Signature: _____
Dr. Angela Forsythe, MD — Cardiology | License #: MA-MD-49733 | Date: May 16, 2025
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